CANNABIS



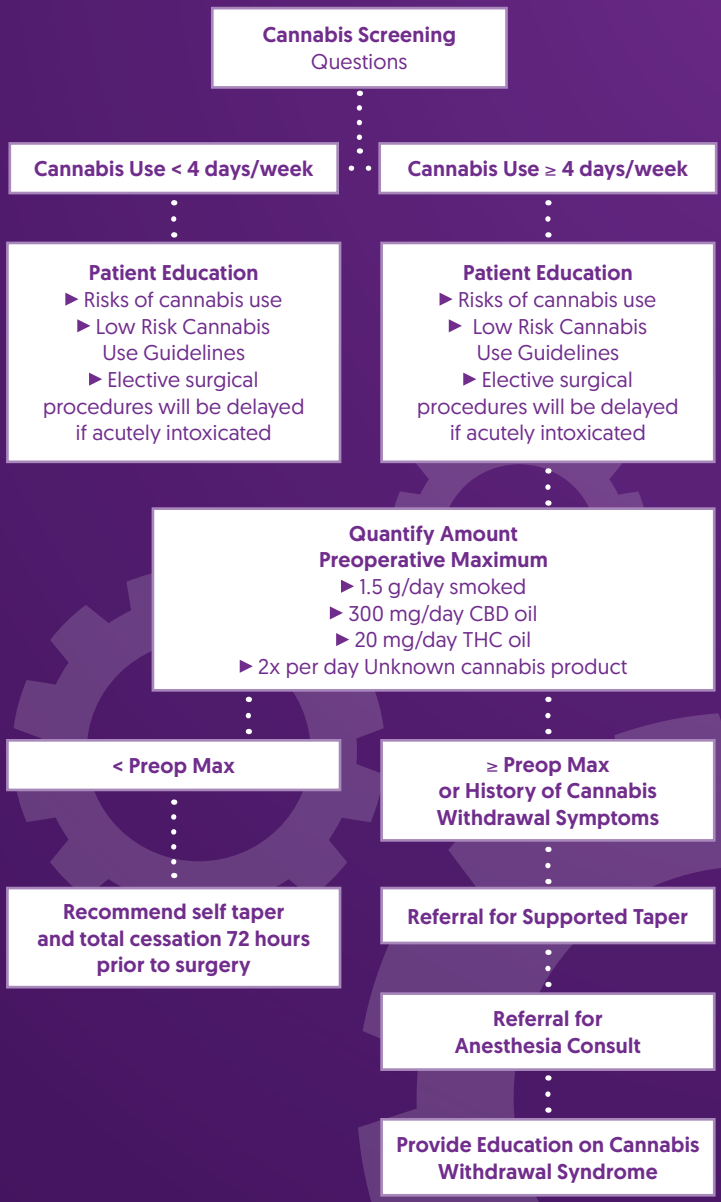
Cannabis (marijuana) use may lead to increased anesthetic requirements, postoperative pain, opioid use after surgery, and nausea and vomiting. [1-3]

Smoking cannabis increases the risk of pulmonary complications, cardiovascular complications (including postoperative myocardial infarction), and in-hospital mortality; it may also increase airway irritation, carboxyhemoglobin, and reduce oxygen-carrying capacity, similar to conventional cigarette smoking. [3-5]

Screening Tools

Cannabis Screening Questions:

- Do you use Cannabis? If Yes:
  - How often?
  - How much do you use?
  - How are you using it? (e.g., smoking, vaping, tincture/oil, edibles, cream)
  - Have you ever had symptoms like headaches, anxiety, poor sleep, or stomach pain when you stopped using cannabis for a day or two (Cannabis Withdrawal Syndrome)?


Cannabis Screening
Questions
Cannabis Use < 4 days/week
Cannabis Use ≥ 4 days/week
Patient Education
▶ Risks of cannabis use
▶ Low Risk Cannabis Use Guidelines
▶ Elective surgical procedures will be delayed if acutely intoxicated
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Quantify Amount
Preoperative Maximum
▶ 1.5 g/day smoked
▶ 300 mg/day CBD oil
▶ 20 mg/day THC oil
▶ 2x per day Unknown cannabis product
< Preop Max
≥ Preop Max or History of Cannabis Withdrawal Symptoms
Recommend self taper and total cessation 72 hours prior to surgery
Referral for Supported Taper
Referral for Anesthesia Consult
Provide Education on Cannabis Withdrawal Syndrome

Patient Education

- Advise patients on perioperative risks of cannabis use [1,2]
  - "Cannabis use puts you at higher risk of heart attacks and lung complications as well as nausea and vomiting and increased pain after surgery."
  - "It is best to stop using cannabis for at least 72 hours before surgery. This does not apply to creams."
- Advise patients that elective surgical procedures will be delayed if they are acutely intoxicated
- Discuss Canada's Lower Risk Cannabis Use Guidelines: [6]
  - Choose low-strength products, such as those with a lower THC content or a higher ratio of CBD to THC
  - Avoid using synthetic cannabis products
  - Smoking cannabis (for example, smoking a joint) is the most harmful way of using cannabis because it directly affects your lungs
- Refer to online patient resources

Recommend Self Taper

- Lower amount every day over 7 days with total cessation 72 hours prior to surgery
- "If you use a lot of cannabis on a daily basis you should slowly decrease the amount you use every day until you can stop for 72 hours without feeling unwell."
- Slow taper if experiencing symptoms of Cannabis Withdrawal Syndrome (CWS)

Prehabilitation and Optimization Recommendations

Referral for Supported Taper	⚙ Consider tapering to a goal of less than the recommended preoperative maximum (rather than complete cessation 72 hours preop) ⚙ Consider primary care, Addiction Medicine, or RACE line support
Referral for Anesthesia Consult	⚙ To develop effective pain management strategies and explore options for anesthesia and postoperative pain control, especially in moderate to severely invasive surgery
Provide Education on Cannabis Withdrawal Syndrome [9]	⚙ Usually begins 1-2 days after cessation and lasts up to 3 weeks ⚙ Symptoms include: headaches, anxiety, poor sleep, stomach pain ⚙ Treatment: • Supportive counseling and psychoeducation • Consider referral for supported taper or suggest slower taper (10% reduction per day) • No medications are approved for medically assisted withdrawal, though some are used 'off-label' in clinical practice